

State of Alaska – Office of the Governor — 34th Alaska State Legislature – Second Session
DEPARTMENT REVIEW – PASSED LEGISLATION

BLUE SHEET

Blue Sheets are a department's final bill analysis and provides the Commissioner an opportunity to weigh in on legislation that has passed, with a recommendation to the Governor on action that may be taken– to enact the bill, to veto the bill, or to allow the bill to become Law without signature (LWOS). Much of the information requested in the Blue Sheet can be taken from the department's bill analyses (BA), which were initially submitted at the time the bill was introduced and updated throughout the legislative process.

Blue sheets are due to the GLO three days after the enrolled bill appears on basis, or sooner by request.

Department: DCCED

Date: 5/12/2026

Bill Number: SB 89

Short Title (as it appears on BASIS): Physician Assistant Scope of Practice

What action do you recommend the Governor take?

Select one.

☐ SIGN ☐ VETO ☒ LWOS

Action Justification – Why do you recommend the above action?

Please be specific.

This bill was amended based on a compromise made between the Physician Assistant (PA) association and the Alaska State Medical Association (ASMA), resulting in support from ASMA that didn't previously exist. After years of searching for a compromise, this is a solution to PA scope of practice that PAs and physicians agree on. It amends the PA laws to allow more flexibility (i.e., removed barriers) for PAs; and now that this legislation is supported by both PAs and physicians, we'd recommend signing as it aligns with the Governor's priorities by expanding access to care and removing barriers for PAs to practice. Per testimony on this bill and the previous version in the 33rd legislature, some PAs are spending in excess of \$50K a year to pay a physician to be their collaborator. If a PA is unable to find a collaborating physician, they currently must close their practice. These issues limit PAs' ability to run their business and practice, add to health care costs in Alaska, and limit access to care. Additionally, this legislation puts the scope of practice for PAs into statute (it is currently only in regulation so, subject to change with a different composition of the State Medical Board).

What does this Bill do?

In addition to providing a succinct bill summary, please also consider the following: Why is this bill needed? How does it specifically change department programs? What is the anticipated impact on the public, the private sector? Does this bill align with the priorities of Governor Dunleavy?

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This legislation amends the scope of practice and collaborative practice agreements for physician assistants (PAs), allows PAs to work without collaborative practice agreements when working in certain facilities, and establishes the qualifications for PA licensure in statute (as those are currently established by the State Medical Board in regulation).

This final version is based on a compromise between physicians and PAs. It provides that PAs are able to provide medical services they have completed the necessary education, training, and experience to provide; and that they are responsible for the care they provide to patients. PAs that do not need a collaborative agreement with a physician are those who are practicing at a facility directed by a licensed physician, osteopath, or podiatrist; a health care facility licensed by DOH under AS 47.32; a federally operated health care facility; a federally qualified health center or rural health clinic; a military and veterans' facility; or another facility if approved by the board upon application by the PA. PAs that are practicing outside of these locations must have entered into a written agreement with an Alaska-licensed physician or physician exempt from Alaska licensure requirements.

PAs in Alaska have been advocating for an expanded scope of practice for many years but struggled to reach an agreement. The Alaska State Medical Association had been avidly opposed until the final version of the bill, which has removed the controversy surrounding this topic.

The bill has little impact on department operations. It will impact the public by likely making health care more accessible as a barrier to practice is removed for many PAs.

Detailed Sectional Analysis

The full sectional analysis of the bill should be completed by the lead agency. Other departments should include an analysis for sections of the bill affecting their agencies.

**-- Sections 1-18 & 25 all amend AS 08.64: Medicine --
-- Other sections amend various chapters of law --**

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|-----------|--|
| Section 1 | Amends AS 08.64.010 – <i>creation and membership of the board</i> – related to the physician assistant (PA) seat on the State Medical Board (“board”), to require the PA to be “licensed in the state” rather than “licensed under AS 08.64.107.” |
| Section 2 | <p>Repeals and reenacts AS 08.64.107 – <i>scope of practice of physician assistants; collaborative agreements</i>:</p> <ul style="list-style-type: none">• A PA may provide any medical services that they are qualified to provide by education, training, and experience and competent to perform, including the services specifically outlined.• A PA is responsible for the care provided to a patient by the PA.• Except as provided below, a PA can only practice in the state under a written collaborative agreement entered into with a physician licensed under AS 08.64 or a physician exempt from licensure requirements under AS 08.64.370. The agreement must describe how the collaboration will occur. |

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When a PA applies for a license or license renewal, they must include a written attestation that the PA is practicing under an agreement, and a copy of the agreement must be provided to the board upon request.

- A PA can practice without a collaborative agreement if the PA is practicing at a facility directed by a licensed physician, osteopath, or podiatrist; a health care facility licensed by DOH under AS 47.32; a federally operated health care facility; a federally qualified health center or rural health clinic; or a military and veterans' facility. The board can also authorize a PA to practice at a facility that does not fall into one of these categories without a collaborating agreement upon the PA's application for such authorization.

- Section 3 Adds a new section AS 08.64.206 – *qualifications for physician assistant applicants* – to require PAs to meet the qualifications in AS 08.64.200(a)(3)-(5) – which requires a list of negotiated settlements or judgments; not having had a license to practice medicine suspended or revoked; and receiving education in pain management, opioid use, and addiction – and submit:
- Certificate of graduation obtained from a PA program accredited at the time of graduation by:
 - If the applicant graduated before January 1, 2001: the American Medical Association's Committee on Allied Health Education and Accreditation or the Commission on Accreditation of Allied Health Education Programs; or
 - If the applicant graduated after January 1, 2001: the Accreditation Review Commission on Education for the Physician Assistant.
 - Proof of current certification issued by the National Commission on Certification of Physician Assistants (NCCPA).
 - Proof of passing the PA national certifying examination offered by the NCCPA.
- Section 4 Adds a new subsection (d) to AS 08.64.230 – *license granted* – to require the board or board's executive secretary (called an "executive administrator" or EA internally) to issue a PA license if the PA passes the exam and meets the requirements of AS 08.64.206 (created by Section 3) and AS 08.64.255 (*interviews*) so they can practice as permitted under AS 08.64.107 (Section 2).
- Section 5 Amends AS 08.64.250(a) – under *license by credentials* – to add PAs to the professions for which the board can waive the examination requirement and license by credentials if the PA meets the requirements of AS 08.64.206 (created by Section 3), submits proof of continued competence, pays the required fees, and has an active license from another state or passed an exam as specific by the board in regulations.
- Section 6 Amends AS 08.64.270(a) – under *temporary permits* – to allow PAs to be issued temporary permits.

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- Section 7 Amends AS 08.64.275(a) – under *temporary permit for locum tenens practice* – to allow PAs to be issued locum tenens temporary permits.
- Section 8 Adds a new subsection (g) to AS 08.64.275 – *temporary permit for locum tenens practice* – to require a PA applying for a locum tenens temporary permit to pay the required fee and meet the requirements of AS 08.64.206 (created by Section 3) and AS 08.64.279 (*interview for permits*), as well as submit evidence of holding a license to practice in a U.S. state/territory or providence/territory of Canada.
- Section 9 Amends AS 08.64.312(c) – under *continuing education requirements* – to allow the board to exempt a PA from the continuing education (CE) requirements if the PA gives satisfactory evidence that they’re unable to comply with the requirements because of extenuating circumstances. Restrictions on how many and which CEs can be waived remain as they exist currently for physicians, osteopaths, and podiatrists.
- Section 10 Amends AS 08.64.326(a)(12) – under *grounds for imposition of disciplinary sanctions* – to change “physician” to “licensee,” so the section applies to all licensees under the State Medical Board (physicians, osteopaths, podiatrists, and PAs).
- Section 11 Amends AS 08.64.334 – *voluntary surrender* – to prohibits a voluntarily surrendered PA license from being returned to a PA if the surrender resulted in the dropping or suspension of civil or criminal charges against the PA. (This law exists now for physicians only.)
- Sections 12-14 Amend AS 08.64.336(a), (e), and (f) – under *duty of physicians and hospitals to report* – to make the duty to report applicable to PAs, not just physicians.
- Section 15 Amends AS 08.64.360 – *penalty for practicing without a license or in violation of law* – to remove the exception for physician assistants.
- Section 16 Amends AS 08.64.370 – *exceptions to application of chapter* – to make the exceptions to licensure requirements that are currently only applicable to physicians, osteopaths, and podiatrists extend to PAs.
- Section 17 Amends AS 08.64.380(6) – under *definitions* – to amend the definition of “practice of medicine” or “practice of osteopathy” by clarifying how a title can be used or publicly displayed; add use of the title “physician assistant” or “P.A.,” and to clarify that these definitions do not include use of the title “doctor” by a person who holds a doctorate degree who is not a physician or osteopath and who clearly communicates that they are not a physician or osteopath.

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- Section 18 Amends AS 08.64.380(7) – under *definitions* – to add to the exception in the “practice of podiatry” that state a patient shall be concurrently referred to a physician or osteopath for the treatment of the systemic disease itself so that patients can also be referred to PAs.
- Section 19 Amends AS 11.71.900(20) – under *Criminal Law: controlled substances: definitions* – to amend the definition of “practitioner” to include PAs.
- Section 20 Amends AS 13.52.390(31) – under *Health Care Decisions Act: definitions* – to state PAs are licensed under AS 08.64, not AS 08.64.107 specifically. Conforming change.
- Section 21 Amends AS 18.08.089(a) – under *Emergency Medical Services: authority to pronounce death* – to state PAs are licensed under AS 08.64, not AS 08.64.107 specifically. Conforming change.
- Section 22 Amends AS 21.07.010(b) – under *Patient Protections under Health Care Insurance Policies: patient and health care provider protections* – to add a subsection so the contract between a participating health care provider and insurer that offers a health care policy cannot contain a provision that imposes a practice, education, or collaboration requirement on PAs that’s inconsistent with, or more restrictive than, the requirements imposed under AS 08.64 or a State Medical Board regulation.
- Section 23 Amends AS 23.30.395(3) – under *Alaska Workers’ Compensation Act: definitions* – to remove the reference that a PA must be under the supervision of a licensed physician or osteopath. Conforming change.
- Section 24 Amends AS 33.30.901(10) – under *Prison Facilities and Prisoners: definitions* – to remove the reference that a PA must be under the supervision of a licensed physician or osteopath. Conforming change.
- Section 25 Repeals AS 08.64.170(a)(1) – under *license to practice medicine, podiatry, or osteopathy* – which is the licensure exemption that allows PAs to examine, diagnose, or treat persons under the supervision, control, and responsibility of a physician. Conforming change.

Companion or Similar Bills

Please note companion legislation, the sponsor of the companion legislation, and identify any “rider” bills or provisions from other legislation that was included in this final bill. Please include the name of the sponsor of the “rider bill” or provision. If it was added as an amendment in a committee – please indicate the name of the legislator that sponsored the amendment.

None.

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Support/Opposition of the Bill

Please identify names, organizations, titles, and phones numbers of those who testified in support or in opposition of this bill.

Support:

- Alaska State Medical Association, Rhene Merkouris, President: (907) 562-0304
- Alaska Primary Care Association, Joshua Gilmour, Board Chair: (907) 929-2722
- Alaska State Medical Board, Brent Taylor, Chair: medicalboard@alaska.gov

Opposition:

- *Unknown, there were many letters of opposition initially, but they seem specific to the previous versions of the bill and not this final version that was based on compromised and resulted in ASMA changing from an opposed to supportive position.*

Bill Signing ceremony suggestions (if applicable)

Please provide information for consideration for a bill signing ceremony. Information that is helpful includes: a specific conference, event, venue, date, and the names and contact information of stakeholders who should be considered for an invitation to the bill signing ceremony.

Suggested event:

- 2026 All-Alaska Medical Conference – October 14-16, 2026, at the BP Energy Center in Anchorage
- Alaska Hospital and Healthcare Association (AHHA) Annual Conference – September 15-16, 2026, at the Alyeska Resort in Girdwood

Stakeholders to consider inviting:

- State Medical Board members – contact through DCCED-Division of Corporations, Business, and Professional Licensing Director Sylvan Robb: sylvan.robb@alaska.gov or (907) 465-2524
- Alaska State Medical Association President, Rhene Merkouris: (907) 562-0304
- Alaska Primary Care Association Chair, Joshua Gilmour: (907) 929-2722

DocuSigned by:

Commissioner's Signature: _____ Date: 5/18/2026
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I have reviewed the above information and provided it to the Governor's Legislative Office for consideration of an action by the Governor on legislation passed during the second session of the 34th Alaska State Legislature.

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